

auracare

Revolutionizing Frontline Care

THE PROBLEM

Personalized medicine needs the full picture, that infrastructure doesn't exist.

SHORT CONSULTATIONS

5 Minutes

The average duration of patient consultations in more than half of the world.¹

DATA DEFICIT

44%

The percentage of consultations where a lack of patient data harms the final diagnosis.²

FRAGMENTED NARRATIVE

Daily life

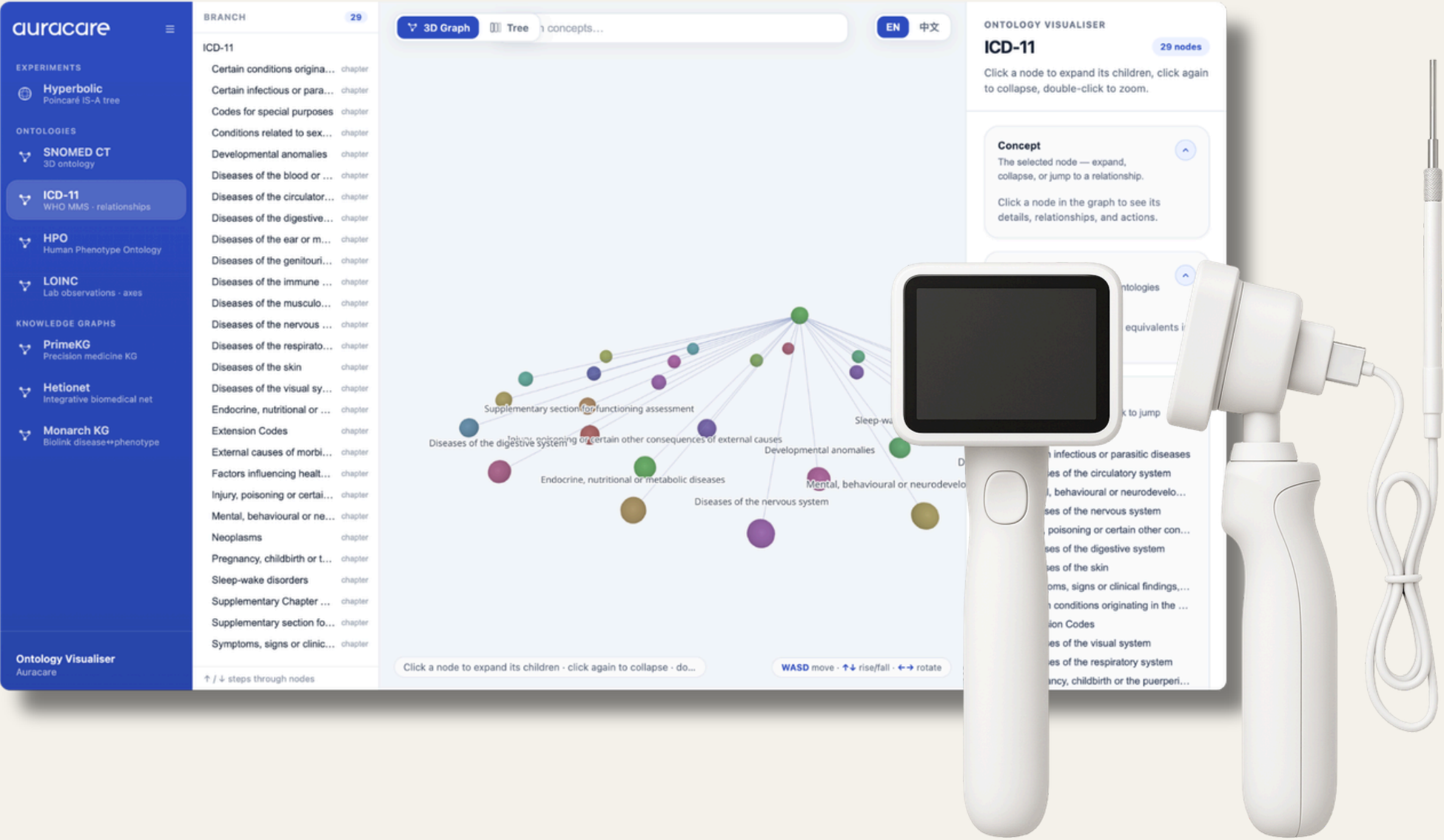
The EHR is a series of snapshots: long gaps with no context in between.

Consultations are just a snippet. Data is incomplete. The full picture is obscured.

MEET AURACARE

A full support system for patient care.

Reasoning through encoded patient information by cross-referencing social history, vitals, medical testing, and the live consultation to produce a full diagnostic in the background; while the clinician focuses on the patient.



STRUCTURE

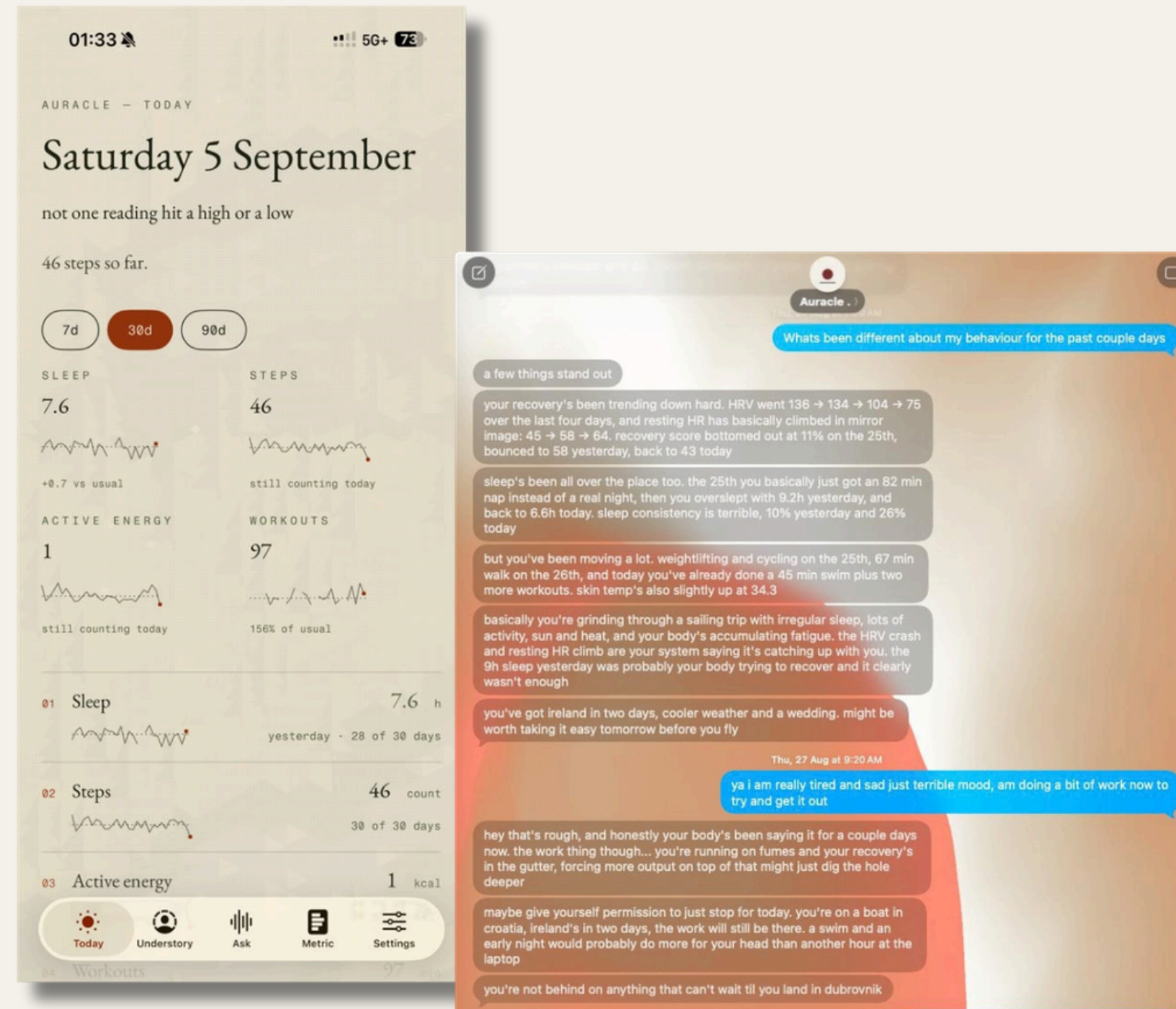
The Core Offering

DATA AGGREGATION ENGINE Encodes daily life A patient portal that aggregates user data and encodes clinical findings using SNOMED CT standards.	CLINICAL DECISION SUPPORT SYSTEM Reasons with the data Our data engine analyzes patient data and cross-references it with established clinical guidelines to propose accurate differential diagnoses.	PROPRIETARY HARDWARE Supports the exam Stethoscope, BP monitor, otoscope, all built in-house. We tailor our devices to translate vitals into our reasoning core.
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THE WEDGE • STAGE 1

We're starting at the patient.

Stage 1 is Auracare's **data aggregation engine**, shipped as a patient-oriented portal for clinics to manage customer context and history pre-consult. It learns a person's daily life from existing online connections and external information, encoding everything into SNOMED CT, fed directly to our CDSS.



THE ENTRY POINT

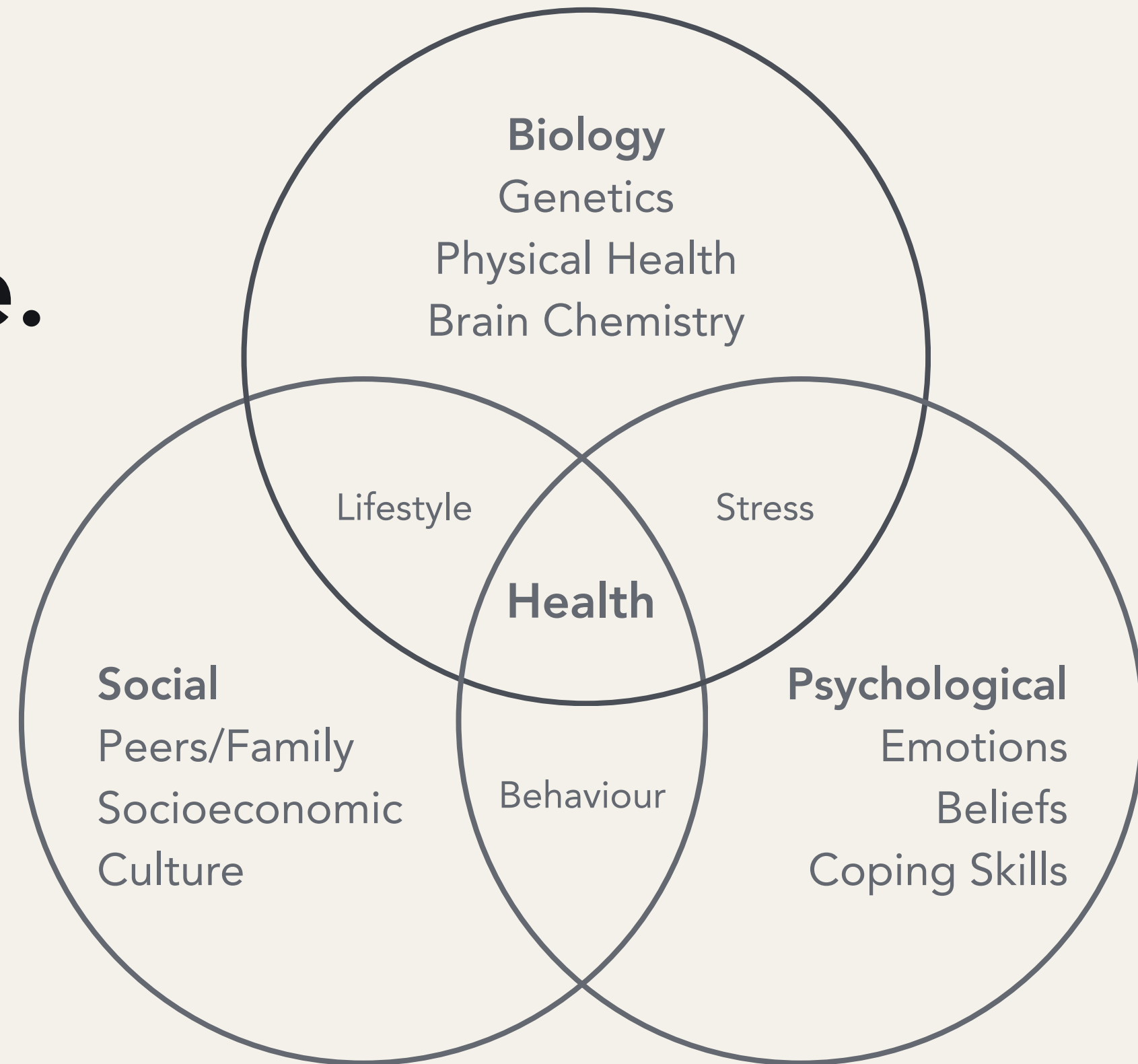
Consultations don't paint the full picture.

BIOPSYCHOSOCIAL MODEL

90%

of a person's health is influenced by non-medical factors, excluding genetics.³

Our Data Aggregation Engine completes the patient history.



THE STAGES

CDSS: The Full Picture

STAGE 1

Connectors

A full lifestyle picture shared by the patient and encoded continuously.



Data Aggregation Engine

Collating patient data from all sources:
The complete picture.



Third Party Information

Prescriptions, consultations, and imaging from a patient's past check-ups.

STAGE 2

Live Consultation

Recorded and transcribed with detail and encoded.



Deterministic Inference Core

Reasons over all data through NICE / MIMS to create a personalized care plan.



Proprietary Hardware

Normalized signals feeding into the core, sourced by our in-house hardware.

STAGE 3

Integrated Documentation

Documentation standardized to the hospital's existing systems.

Ranked differentials

Ordered by likelihood and evaluated against the patient's own history.

Supply Optimization

Reducing stock / workforce wastage and tailoring care selection.

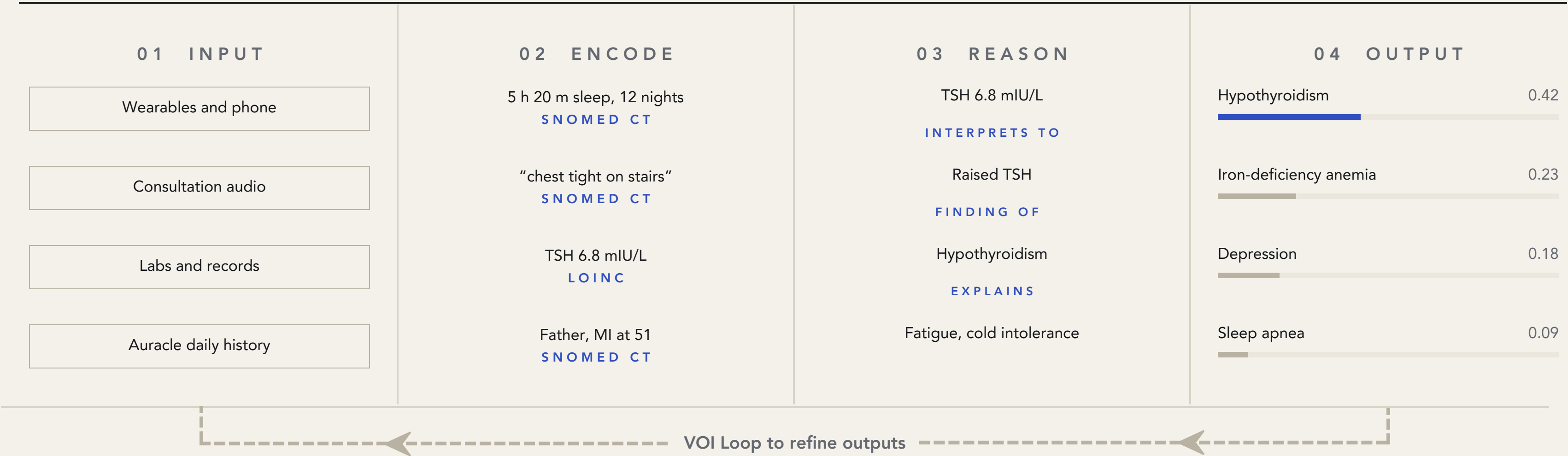
ENCODING
.....
SNOMED CT · ICD-11
LOINC · HPO



Glass box, not a Black Box

Full data transparency over our reasoning model.

Data Aggregation Engine → Deterministic Inference Core → Outputs; all within a feedback loop.



OUR OWN HARDWARE

We build the hardware, so the data comes easy.

Social History is only one piece of the puzzle. A full clinical decision needs measurement.

We manufacture our own examination devices through OEM partnerships in China,
so vitals arrive normalized and coded to our need without relying on third-party hardware.

Recording stethoscope Captures heart and lung activity as data the core can read.	Blood-pressure monitor Clinical-grade blood pressure readings, streamed straight into the reasoning core.	Otoscope Ear canal/drum imaging is captured and passed to the core automatically.
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Three devices built, more on the way. Certification targeted for 2027.

INITIAL TARGET CUSTOMER

The Target

Private outpatient healthcare conglomerates that own and run networks of outpatient clinics; brands and specialties under one umbrella entity that overlooks ownership and operations.

Our beachhead will be focused on the East Asian region, Hong Kong and China, using our established relationships with EC Healthcare and the Chinese Academy of Sciences for clinical trialing.

More patients per clinician **THROUGHPUT**

- 1 The record writes itself as the consultation happens. Time returned per visit becomes more patients per hour.

One patient across every clinic **REFERRALS**

- 2 A diagnosis today is whatever word that doctor chose that day, so nothing transfers between brands. Coded records make internal referral work.

A structure to build on **AUTOMATION**

- 3 Nothing can be automated on notes that vary by author. Encoding is the precondition for stock optimization, audit and every tool introduced.

MONETIZATION

Direct to Digital

API access for clinics; generalized features with basic support, no tailored integration into existing systems

Direct to Service

Sold to the groups that own the clinics, priced on the clinician time it saves.

TRACTION

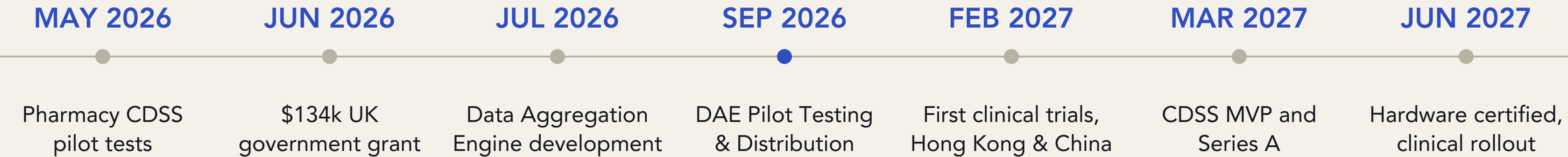
Building Momentum

Funding, partnerships and clinical pilots already in place.

UK PHARMACY PARTNERSHIPS, CDSS PILOT	28
UK GOVERNMENT FUNDING AWARDED	\$134k
ANGEL INVESTMENT, HONG KONG	\$400k
CLINICAL TRIAL, EC HEALTHCARE	Dec 2026
CLINICAL TRIAL, CHINESE ACADEMY OF SCIENCES	Feb 2027
HARDWARE PARTNERSHIPS, CHINESE OEMs	2
Completed Founders Factory Healthcare: Hospital to Community cohort and secured 3-year Vanta contract for 5 regulatory frameworks (ICO and CE).	

THE JOURNEY

The Road to the Clinic



OPPORTUNITY

A Large Clinical Market

\$30B

Total addressable market⁴

WHO Workforce: 12.7 million medical doctors worldwide × \$2,400 per clinician per year.

\$11B

Serviceable obtainable market

The segments our launch markets and clinical route actually reach.

COMPARABLES

OpenEvidence
\$12B valuation

Bayesian Health
~\$40m valuation

THE TEAM

Clinical, engineering and manufacturing



Tanush Pandey

CO-FOUNDER · CEO & CMO

Medicine at Imperial College London. Leads clinical strategy, safety, and research, along with HCP relations.



Stephen Okita

CO-FOUNDER · CTO

Computer science at UC Berkeley. Sets the architecture and product direction; business relations in San Francisco.



Hinlun Chen

CO-FOUNDER · COO

PPE at LSE; runs an OEM with two factories. Leads operations and partnerships in Hong Kong and China.



Julius Chu

CPO

Computer science & Entrepreneurship at NYU. Leads product, design system, the consumer experience.



Karam Elradie

HEAD OF ML

Leads machine learning (PhD): the neuro-symbolic core and graph models behind our reasoning.



UC Berkeley



THE ASK

We are raising a seed round

\$2.21M

12-month runway

\$2.01M deployed, plus 10% contingency.

Team	6–7 personnel, \$50–200k per head	\$910k
Clinical & regulatory	Class IIa/IIb trials, CDSS studies, hardware certification	\$500k
Operating & G&A	IP and patents, legal, insurance, admin, travel	\$300k
Marketing	Social, online and content-led customer acquisition	\$200k
Compute & data	AI training runs, processors and data	\$100k
TOTAL DEPLOYED		\$2.01M

auracare

GET IN TOUCH

Be a pioneer.

We are raising a seed round to build the core engine,
certify the hardware, and take Auracare to clinics.

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Sources

- 1 Irving G, et al. International variations in primary care physician consultation time: a systematic review of 67 countries. BMJ Open 2017;7:e017902.
- 2 Smith PC, et al. Missing Clinical Information During Primary Care Visits. JAMA 2005;293(5):565-571.
- 3 Hayes TO, Delk R. Understanding the Social Determinants of Health. American Action Forum 2018.
- 4 Boniol M, et al. The global health workforce stock and distribution in 2020 and 2030. BMJ Global Health 2022;7:e009316.

Market figures from the OpenEvidence and Bayesian Health valuations are drawn from published funding rounds and press coverage.
